

PSA testing

PCRMP 2022, NICE NG12 2015 (updated 2021)



Red Whale

GEMS

Guidelines & Evidence Made Simple

Man aged $\geq 50y$ requests PSA test

Does this man have symptoms?

- Lower urinary tract symptoms (LUTS), e.g. nocturia, frequency, hesitancy, urgency or retention
- Erectile dysfunction
- Visible haematuria (in the absence of UTI)

NO

YES

Discuss pros and cons of PSA testing
Offer appropriate written material

Assess symptoms and follow NICE NG12

- Consider offering DRE and PSA test
 - Refer on cancer referral pathway if prostate feels malignant on DRE
 - Consider cancer referral pathway if PSA raised
- NOTE: visible haematuria may also warrant cancer referral on renal/bladder pathway – see haematuria GEMS

Decides against test

Decides to have test

PSA raised

PSA not raised

End of pathway – can revisit in the future

End of pathway – can revisit in the future

Refer according to local pathways

- PCRMP 2022 does not state what counts as a 'raised PSA' in the context of screening, though trials used $\geq 3.0\text{mcg/l}$ as the cut-off
- In many areas, we will refer according to NICE NG12 age-related cut-offs (see below for more details), but we should note that the evidence that sits behind these is based on men with symptoms, NOT screening (asymptomatic) PSA

Can raise PSA

Can lower PSA

Benign prostatic hypertrophy
Older age
Prostatitis
Recent ejaculation
Prostate cancer

Obesity
Drugs:
• 5ARIs (e.g. finasteride)
• Aspirin
• Statins
• Thiazides

Practicalities for doing a PSA test

Men should not have:

- An active urine infection at the time of the test or within the previous 6 weeks
- Ejaculated in the previous 48 hours
- Exercised vigorously, e.g. cycling, in the previous 48 hours
- Had urological intervention, e.g. prostate biopsy/cystoscopy, in the previous 6 weeks

The test must reach the lab in time for serum to be separated within 16 hours (*or be spun down in practice*)

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